

1. Administrative & Study Identification

Full Study Title: A Phase III, Randomized, Double-Blind, Placebo-Controlled Study of Atezolizumab Plus Tiragolumab in Combination With Paclitaxel and Cisplatin Compared With Paclitaxel and Cisplatin as First-Line Treatment in Patients With Unresectable Locally Advanced, Unresectable Recurrent, or Metastatic Esophageal Squamous Cell Carcinoma **Short Title/Acronym:** Atezolizumab + Tiragolumab + Chemotherapy in 1L Esophageal Carcinoma **Protocol Number:** 863295936506667857 **NCT Number:** NCT04540211 **Sponsor Name:** Roche **Investigational Product:** Atezolizumab, Tiragolumab, Paclitaxel, Cisplatin (Trade Name: Platinol, ATC Code: L01XA01) **Phase of Development:** Phase III **Medical Monitor:** Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy of atezolizumab plus tiragolumab in combination with paclitaxel and cisplatin (PC) compared with atezolizumab matching placebo plus tiragolumab matching placebo plus PC as first-line treatment, measured by Overall Survival (OS) and Progression-Free Survival (PFS). **Secondary Objectives:** To evaluate Investigator-Assessed PFS, IRF-Assessed Confirmed Objective Response Rate (ORR), and Investigator-Assessed Confirmed ORR.

2.2 Background & Rationale To address the clinical need for improved first-line therapeutic options in participants with unresectable locally advanced, unresectable recurrent, or metastatic esophageal carcinoma (EC). Combining immune checkpoint inhibitors (anti-PD-L1 atezolizumab and anti-TIGIT tiragolumab) with standard chemotherapy (paclitaxel and cisplatin) aims to enhance anti-tumor immune responses and improve overall survival outcomes compared to standard chemotherapy alone.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled (Atezolizumab matching placebo + Tiragolumab matching placebo + PC) **Blinding:** Double-blind **Randomization:** Randomized 1:1 (Arm A: Active combination vs. Arm B: Placebo combination)

3.2 Planned Enrollment & Duration Target \$N\$: 461 evaluable patients **Number of Sites:** Multicenter/Global **Estimated Study**

Start: Unspecified **Estimated Primary Completion:** Study is COMPLETED

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histologically confirmed Esophageal Carcinoma (EC) with unresectable locally advanced, unresectable recurrent, or metastatic disease. 2. Measurable disease per Response Evaluation Criteria in Solid Tumors version 1.1 (RECIST v1.1). 3. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1, with adequate hematologic and end-organ function. 4. Female participants must be willing to avoid pregnancy and refrain from donating eggs during the treatment period and for 90 days after the final dose. 5. Male participants with partners of childbearing potential must commit to the use of two methods of contraception and must not donate sperm for the study duration and 90 days after the final dose.

4.2 Exclusion Criteria 1. Palliative radiation treatment for EC within 4 weeks prior to initiation of study treatment, or evidence of complete esophageal obstruction not amenable to treatment. 2. Symptomatic, untreated, or actively progressing central nervous system (CNS) metastases or leptomeningeal disease. 3. Prior treatment with CD137 agonists or immune checkpoint blockade therapies (anti-CTLA-4, anti-TIGIT, anti-PD-1, and anti-PD-L1 therapeutic antibodies). 4. Uncontrolled tumor-related pain, uncontrolled pleural effusion, pericardial effusion, or ascites requiring recurrent drainage procedures. 5. Active or history of autoimmune disease, or immune deficiency. 6. History of idiopathic pulmonary fibrosis, organizing pneumonia, drug-induced pneumonitis, or idiopathic pneumonitis, or evidence of active pneumonitis. 7. Malignancies other than EC within 2 years prior to screening with a negligible risk of metastasis or death adequately treated with expected curative outcome. 8. Severe infection within 4 weeks prior to initiation of study treatment, any active infection impacting patient safety, positive test result for HIV, or active hepatitis B or hepatitis C. 9. Treatment with any investigational therapy prior to initiation of study treatment. 10. Poor peripheral venous access. 11. Prior allogeneic stem cell or solid organ transplantation. 12. Concurrent participation in another therapeutic clinical trial.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Induction phase consisting of Atezolizumab + Tiragolumab in combination with Paclitaxel and Cisplatin. Following the induction phase, participants continue maintenance therapy with Atezolizumab + Tiragolumab (Arm A) or matching placebos (Arm B). **Route of**

Administration: Intravenous (IV) **Duration of Treatment:** Until disease progression or unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care and premedication for chemotherapy regimens. **Prohibited:** Concurrent participation in another therapeutic clinical trial, live virus vaccines, and prior allogeneic stem cell or solid organ transplantation.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Imaging (RECIST v1.1), Labs
Baseline	Day 0	Randomization, First Dose (Induction Phase)
Interim	Per Cycle	Treatment Administration, Safety Labs, Efficacy Assessment (Tumor Scans)
End of Study	Follow-up	Disease Progression, Final Vital Signs, Survival Follow-Up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: 1) Overall Survival (OS); 2) Independent Review Facility (IRF)-Assessed Progression-Free Survival (PFS). **Measurement Method:** Survival tracking and Central Radiological Review using RECIST v1.1.

7.2 Secondary & Exploratory Endpoints * Investigator-Assessed PFS
* IRF-Assessed Confirmed Objective Response Rate (ORR) *
Investigator-Assessed Confirmed ORR

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous collection via patient interviews, physical exams, and laboratory testing during each cycle. **Serious Adverse Events (SAEs):** Reported within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee to evaluate safety and efficacy signals.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy analyses; Safety Set for participants who received at least one dose of study medication. **Sample Size Justification:** \$N = 461\$, powered to detect statistically significant improvements in OS and PFS. **Handling of Missing Data:** Standard Kaplan-Meier censoring rules for time-to-event endpoints (OS, PFS, DoR).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring visits to ensure protocol compliance and data integrity. **Audit Trail:**

Secure eCRF locking and electronic data clarification forms utilized across all clinical sites.

11. Study Identifiers & Governance

Primary ID: 863295936506667857 **Registry Number:** NCT04540211

Sponsor/Lead Organization: Roche **Study Title:** A Study of Atezolizumab Plus Tiragolumab in Combination With Paclitaxel and Cisplatin Compared With Paclitaxel and Cisplatin as First-Line Treatment in Participants With Unresectable Locally Advanced, Unresectable Recurrent, or Metastatic Esophageal Carcinoma

Official Regulatory Title: A Phase III, Randomized, Double-Blind, Placebo-Controlled Study of Atezolizumab Plus Tiragolumab in Combination With Paclitaxel and Cisplatin Compared With Paclitaxel and Cisplatin as First-Line Treatment in Patients With Unresectable Locally Advanced, Unresectable Recurrent, or Metastatic Esophageal Squamous Cell Carcinoma

12. Clinical Framework (Esophageal Carcinoma Specific)

Primary Condition: Esophageal Cancer (Preferred Name: Malignant neoplasm of esophagus) **Diagnosis Threshold:** Histologically confirmed Esophageal Carcinoma; Unresectable locally advanced, recurrent, or metastatic

Medical Methodology: First-line standard Paclitaxel and Cisplatin (PC) chemotherapy **Optimization Target:** Atezolizumab plus Tiragolumab dual immune checkpoint blockade

13. Study Procedures & Methodology

Management Pathway: First-line oncology induction and maintenance pathway **Education Protocol (HCP):** Intravenous infusion protocols and immune-related adverse event (irAE) management

Education Protocol (Patient): Symptom tracking and prompt reporting of immune-mediated adverse events **Quality Audit Mechanism:**

Independent Review Facility (IRF) RECIST v1.1 tumor assessments

14. Observation & Follow-Up Schedule

Total Duration: Until disease progression, unacceptable toxicity, or death **Onsite Visit Cadence:** Day 1 of each treatment cycle

Remote Monitoring Frequency: Routine safety and survival follow-

up calls **Checklist Review Interval:** Tumor imaging assessments
(e.g., every 6 to 9 weeks per protocol)

15. Sign-off & Approval

Role	Name	Signature	Date		----	----	----	----
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					